MERCY GENERAL HOSPITAL - EMERGENCY DEPARTMENT RECORD
Patient: Morgan Ellis DOB: 04/11/1985 MRN: MG-2210934
Arrival: 01/09/2026 08:37 by ambulance (Metro Fire & Rescue Medic 14)
Attending Physician: Kevin Osei, MD

TRIAGE (08:41, R. Delgado, RN):
Chief complaint: MVC, rear-ended. Neck pain, low back pain, headache.
Pain score: 8/10. Acuity: ESI 3.
BP 144/90 HR 100 RR 18 Temp 98.4 F SpO2 99%
Arrived in cervical collar.

HISTORY OF PRESENT ILLNESS:
40-year-old female, restrained driver, who was stopped in traffic on I-5 this morning when her car was struck from behind by a box truck. She reports her head was thrown back and then forward. She denies loss of consciousness. She complains of posterior neck pain radiating to the left trapezius, low back pain without radiation, and a diffuse headache. She denies vision changes, vomiting, chest pain, abdominal pain, numbness, or weakness. No anticoagulant use.

PAST MEDICAL HISTORY: Seasonal allergies. No prior surgeries.
MEDICATIONS: Loratadine as needed.
ALLERGIES: Penicillin (rash).
SOCIAL: Works as a warehouse shift supervisor. Married, two children. Nonsmoker.

PHYSICAL EXAMINATION:
General: Alert, anxious, in moderate distress due to pain.
Head: No hematoma or laceration.
Neck: Cervical collar in place on arrival. Midline tenderness at C5-C6 on initial exam. Paraspinal
tenderness bilaterally, left greater than right. Range of motion deferred until imaging.
Back: Tenderness over the lumbar paraspinal muscles L3-L5. No midline step-off.
Left shoulder: 6 cm linear abrasion and contusion over the left clavicle and anterior shoulder, consistent with seat belt mark. Full range of motion with pain at end range.
Neurologic: Strength 5/5 in all extremities. Sensation intact to light touch. Reflexes 2+ and symmetric. Gait steady.
Chest/Abdomen: No seat belt sign on the abdomen. Nontender.

ED COURSE:
CT head and CT cervical spine obtained given mechanism and midline tenderness (see radiology reports).
Lumbar spine x-ray: no fracture or listhesis.
Ketorolac 15 mg IV given at 09:20 with partial relief; pain 6/10 on reassessment at 10:05.
After negative CT, the collar was cleared. Repeat exam: paraspinal tenderness persists; active cervical rotation limited to approximately 45 degrees bilaterally due to pain.

DIAGNOSES:
1. Acute cervical strain (whiplash injury), S13.4XXA, due to motor vehicle collision.
2. Acute lumbar strain, S39.012A.
3. Contusion and abrasion of left shoulder, S40.012A.
4. Post-traumatic headache.

DISPOSITION AND PLAN:
Discharged home at 11:40 in stable condition with a friend.
Prescriptions: Cyclobenzaprine 10 mg at bedtime as needed for muscle spasm (#15).
Naproxen 500 mg twice daily with food for 10 days.
Ice for the first 48 hours, then heat. Gentle range of motion as tolerated.
Off work for 3 days; work note provided.
Follow up with primary care provider within 3 to 5 days.
Return precautions: worsening headache, vomiting, confusion, numbness, weakness, or loss of bowel or
bladder control.

The patient's injuries are consistent with the reported mechanism of a rear-end motor vehicle collision.

DISCHARGE INSTRUCTIONS ACKNOWLEDGMENT
Patient verbalized understanding of discharge instructions and return precautions.

Electronically signed: Kevin Osei, MD 01/09/2026 11:52
Nursing discharge: R. Delgado, RN 01/09/2026 11:45